

INTRODUCTION

Prolonged “Nothing by Mouth” (NPO) status contributes to malnutrition, patient discomfort, delayed recovery, and extended hospital stays. Preliminary review at Lakeland Regional Health revealed patients remaining NPO >24 hours without clear indication. Contributing factors included duplicate or conflicting diet/NPO orders and lack of automated EMR alerts. This project seeks to improve patient safety and care efficiency by minimizing unnecessary NPO duration through EMR system redesign and provider education.

HEALTHCARE IMPACT

Unnecessary NPO prolongation can lead to aspiration risk, hypoglycemia, dehydration, and longer length of stay. National standards emphasize the importance of minimizing inappropriate NPO times and promoting timely nutrition resumption. Implementing automated alerts supports safe, efficient, and patient-centered care aligned with institutional quality goals.

METHODS

Baseline Data Collection:

- Retrospective review of inpatient NPO orders to quantify current duration (>24 hrs) and frequency of duplicate/conflicting diet–NPO orders.

Root Cause Analysis:

- Lack of alert mechanisms and EMR safeguards against multiple orders.
- Inconsistent communication between ordering physicians, nursing, and procedural teams.

Intervention Design:

- EMR build to **restrict duplicate/conflicting NPO–diet orders**.
- Automated **24-hour NPO alert** to notify ordering physician and escalate to the attending.
- Provider and nursing **education sessions** to reinforce appropriate NPO use and communication.

Outcome Measures:

- Reduction in prolonged (>24 hr) NPO orders.
- Elimination of duplicate/conflicting orders.
- Increased provider awareness and timely diet advancement.

EXPECTED RESULTS

Decrease in NPO >24 hrs occurrences.

Elimination of duplicate diet–NPO orders.

Improved communication and workflow efficiency.

Enhanced patient satisfaction and safety metrics.

Order Entry → Restriction Validation → 24-hr Timer → Alert → Attending Notification

DISCUSSION

This multidisciplinary QI project emphasizes system redesign to prevent human error and improve clinical efficiency. EMR-based alerts are expected to drive provider accountability and ensure prompt reinitiation of diet orders post-procedure. Collaboration among medical, nursing, and IT teams is essential to sustain success.

CONCLUSION

EMR optimization and automated alerts provide a scalable strategy to minimize unnecessary NPO time, eliminate duplicate orders, and promote patient-centered care. Ongoing data collection will evaluate the system’s effectiveness and inform future hospital-wide implementation.

REFERENCES

A.S.P.E.N. Guidelines for Nutrition
Lakeland Regional Health Internal Data, 2025.